

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 9/13/11	

UUID: F50CE124-918B-43E3-83B1-DC64713DA2A9
TCGA-EL-A3D0-01A-PR

Redacted

ry Case gy Report Pathology

DOB

Sex: M

Physician:

Accession:

Case type: Surgical History

**** Case imported from legacy computer system. The format of this report does not match the original case. ****
**** For cases prior to the section "SPECIMEN" may have been added. ****

DIAGNOSIS

(A) LEFT MODIFIED NECK DISSECTION:

METASTATIC PAPILLARY CARCINOMA OF THYROID IN 9 OF 37 LYMPH NODES
(ONE IN LEVEL I, FOUR IN LEVEL II, AND FOUR IN LEVEL III).

METASTATIC PAPILLARY CARCINOMA OF THYROID IN SOFT TISSUE OR REGION
#2.

(B) TOTAL THYROIDECTOMY, MASS IN LEFT LOBE:

PAPILLARY CARCINOMA OF THYROID INVOLVING LEFT LOBE WITH EXTENSION
IN SOFT TISSUES OF THE NECK. TUMOR AT INKED MARGINS OF EXCISION
(MEASURING 4.8 X 3.0 X 1.8 CM).

Fragment of thymus.

Follicular adenoma in right lobe of thyroid.

1CD-0-3

carcinoma, papillary, thyroid 8260/3

Site: thyroid, NOS C73.9

hw
10/15/11

Entire report and diagnosis completed by:
Report released by:

GROSS DESCRIPTION

(A) LEFT MODIFIED NECK DISSECTION - The specimen consists of a 14.0 x 11.0 x 2.0 cm portion of soft tissue and associated lymph nodes, including a 10.0 x 6.0 x 2.0 cm segment of the left sternocleidomastoid muscle. The specimen is oriented into regional lymph node levels by the surgeon.

One possible level I lymph node, 15 possible level II lymph nodes, 9 possible level III lymph nodes, 12 possible level IV lymph nodes, and 1 possible level V lymph node, are identified. Many of the nodes are cystic and contain hemorrhagic fluid. The largest node, which measures 3.5 cm in maximum dimension, is present in the level III region and is grossly replaced by firm, cream-white tumor. The sternocleidomastoid muscle is grossly free of tumor.

SECTION CODE: A1, one level I lymph node; A2-A8, level II lymph nodes (A2, A3, 1 lymph node; A4, A5, 1 lymph node each; A6-A8, 4 possible lymph nodes each); A9-A13, level III lymph nodes (A9, representative sections of grossly positive lymph node; A10, 2 possible lymph nodes; A11, A12, one possible lymph node each; A13, four possible lymph nodes); A14-A17, level IV lymph nodes (3 possible lymph nodes each); A18, one possible level V lymph node; A19, A20, adipose tissue, level V, representative sections; A21, A22, skeletal muscle, representative sections.

(B) TOTAL THYROIDECTOMY (MASS IN LEFT LOBE) - The specimen consists of a 36.0 gm total thyroidectomy, with the right lobe measuring 4.0 x 3.0 x 1.0 cm, the left lobe 5.0 x 3.5 x 3.0 cm, and the isthmus 1.0 x 0.3 x 0.3 cm.

The left thyroid lobe is hard and irregular, and is almost completely replaced by solid, firm, gritty, cream-white tumor. The tumor measures 4.8 x 3.0 x 1.0 cm and grossly extends to the inked surface of the specimen in many areas. A 0.5 x 0.4 x 0.4 cm firm nodule is present at the inferior pole of the left lobe. The uninvolved thyroid parenchyma immediately superior to the tumor contains a separate, well-circumscribed, 0.2 cm, cream-white nodule.

History Case Pathology Report

Department of Pathology,

DOB:

Sex: M

Physician:

received

Te
Pathologist

Accession:

Case type: Surgical History

The right thyroid lobe is largely unremarkable and has a firm, meaty red cut surface. A 0.4 x 0.3 x 0.3 cm tan-white nodule is present at the mid-portion of the lobe. Another 0.3 x 0.3 x 0.2 cm nodule is noted at the inferior pole of the right lobe.

INK CODE: Black-external surface of specimen.

SECTION CODE: B1, nodule, mid-portion, right lobe; B2, nodule, inferior pole, right lobe; B3-B5, right lobe, representative sections; B6, isthmus; B7, nodule, inferior pole, left lobe; B8-B15, tumor, left lobe, representative sections (superior to inferior); B16, left lobe, 0.2 cm nodule superior to main tumor.

SNOMED CODES